PRODUCT PROPOSAL

Care gaps closed in workflow

AI-powered care gap alerts and patient re-engagement, built on the surface Yosi already owns.

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WHERE THIS STARTS

The rails for this already exist.

This is not a new direction. It is a product assembled from surfaces Yosi already owns.

ALREADY SHIPPED

- Bidirectional EMR integration, in production
- Pre-arrival attention from the patient
- Two-way texting and self-scheduling
- A patient relationship that starts before the visit

WHAT IS MISSING

- Nothing decides which gap is worth a patient's attention
- No plain-language layer between chart and patient
- No path from an open gap to a booked appointment
- No measurement that separates lift from seasonality

The delivery mechanism is built. What is missing is the decision layer on top of it.

THE WEDGE

Everyone can compute the gap. Almost nobody can close it.

Detection has been commodity for a decade. The bottleneck was never knowing.

Population health platforms	Produce a list. Someone has to work it.	**Staff capacity**
EMR best-practice alerts	Fire at the clinician mid-visit.	**Alert fatigue**
Outreach and campaign tools	Reach a patient who is not paying attention.	**Nobody answers**
Yosi	Surfaces the gap while the patient is mid-flow, then books it.	**Already engaged**

The failure mode of every competitor is a human who runs out of time. Yosi's asset is the patient's attention.

SEQUENCING

Three surfaces. Start with the cheapest one.

Each one reuses rails that already exist. None requires a new patient channel.

1

Pre-arrival intake

Patient is mid-intake for an appointment that already exists. Gap appears, one tap adds it to that visit.

Highest conversion, zero new outreach, lowest regulatory surface. Ship first.

2

Post-visit

Order placed and never scheduled. The most common leak in every practice.

Reuses self-scheduling. Recovers revenue the practice already earned.

3

Lapsed patient

Nothing on the books. Text that ends in a booked slot, not a call-the-office message.

Highest value per close, hardest consent and channel questions.

INTELLECTUAL HONESTY

Most of this is not AI, and I would not sell it as AI.

Detection is deterministic logic against structured data. Saying otherwise invites a question you cannot answer.

RULES

- Which gap exists
- Eligibility and acuity tier
- Suppression rules
- Who is safe to contact

AI EARNS ITS PLACE

- Plain-language rewrite at the right reading level
- Sequencing when a patient has open gaps
- Channel and timing from prior response behavior

YOSI EDGE

- Which channel a patient actually engages
- When they engage
- Engagement history across practices

A population health vendor can compute the same gap. It cannot know that John answers texts at 6:40pm and never opens email.

DEMO

Working prototype.

Three flows and an inspector that labels every element as rule, AI, EMR read, or Yosi data.

1	**Pre-arrival**	Gap surfaced mid-intake. Added to visit in one tap, no staff task created.
2	**Lapsed patient**	Text with no clinical detail, verification, then a booked slot.
3	**Staff queue**	What closed itself, what needs a human, and every suppression with its reason.

Built to show the workflow and guardrails, not the model.

WHAT COULD GO WRONG

The failure modes are not technical.

Four decisions I would make before writing a requirement, because each one can damage the core product.

Alert fatigue	A budget of one gap per encounter. A declined gap goes quiet for 90 days. Intake completion is a guardrail metric, and if it drops, the feature comes off.
Clinical acuity	Preventive and routine screening only. Abnormal results and anything urgent are not automated. Tiering is a configuration.
PHI in open channels	No diagnoses, results, or clinical details in SMS or email. The message is an invitation. Everything specific sits behind date-of-birth verification.
Staff burden	The queue must shrink, not grow. If the practice ends up working a list, the product has failed regardless of gap closure.

One primary metric, measured against a holdout.

Adherence is the metric everyone reaches for. Yosi cannot observe it, so I would not commit to it.

PRIMARY

Gap closure rate within 90 days

Ten percent of eligible patients receive nothing. Without the holdout there is no way to separate this feature from seasonality, from recall letters, or from the reminders Yosi already sends.
It costs a little closure and buys a number you can put in front of a prospect.

GUARDRAILS

– Intake completion rate must not move
– Opt-out rate per practice
– Inbound calls to the front desk

COMMERCIAL

– Recovered appointments per practice per month
– Gaps closed with no manual interventions

Saying a metric is unobservable is more useful than committing to one you cannot instrument.

WHAT IT IS WORTH

A model with its inputs showing.

Every number here is an assumption. Change any one of them and the answer moves.

8,000	**18%**	**24%**	**+15 pts**
Active patients per practice	Share with an open gap	Baseline closure, 90 days	Assumed lift

Roughly 216 recovered visits per practice per year

Before any quality-program or risk-adjustment value, which is upside I would not underwrite.

PRICING SHAPE

Module on the existing contract, priced per provider. Not per closed gap, which pushes the incentive toward volume.

IF I OWNED THIS

First 90 days.

One specialty, a handful of practices, one gap type. Narrow enough to be real.

1

Weeks 1-3

Pick the target

Five customer conversations and a read of what gap data actually exists per EMR.
Choose one specialty and two gap types.
Write down what would make this a bad idea.

2

Weeks 4-8

Ship flow one

Pre-arrival only, three practices, holdout on from day one.
Rules-based detection, AI on language alone.

3

Weeks 9-12

Prove or stop

Compare gaps closure vs holdout group.
If intake completion moved, that is the finding.

The 90-day deliverable is a decision with evidence behind it.

APPENDIX

Open questions and assumptions

WHAT I DO NOT KNOW

Assumptions I made, and what I would check first.

Data access	I assumed structured gap-relevant fields are readable across Epic, Cerner, and athenahealth at comparable reliability. That is almost certainly false, and the variance probably limits the goals.
Who signs the order	I assumed a clinician approves before anything becomes an order. If practices want it automated, that is a different product with a different risk profile.
Consent posture	I assumed existing text consent covers a clinical nudge. Worth legal/regulatory approval.
Whether practices want it	The value is obvious to a payer. To a small practice it may read as more work for the same reimbursement. That objection would change the pitch, not the product.